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7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **SURESH DIKALE**	**Age :** 51 Y | **Sex :** Male	**SAMPLE COLLECTED AT :** BORA CLINICAL LABORATORY. LASALGAON. NASHIK
Ref By	: **DR. SELF**	**Registered on :** 25-08-2026 06:57 PM	
Reg No	: **HL7201826382 / MH394**	**Received on :** 28-08-2026 10:39 AM	
Barcode	: **HEJ192877**	**Reported on :** 29-08-2026 12:53 PM	**SAMPLE :** Serum
INV	: Allergy Profile Without Nonveg		

ALLERGY PROFILE WITHOUT NONVEG

TEST DESCRIPTION	RESULT	UNITS	RANGES
SERUM IMMUNOGLOBULIN - E (IgE) Method: CLIA	399	IU / ml	0-3 Y : 0 - 46 3-16 Y : 0 - 280 ADULTS-. Y : 0 - 100

ALLERGY SCREENING TEST (BY ELISA)

NORMAL VALUE FOR ALL ALLERGEN IS LESS THEN	**0.35 IU / ml**	**NEGATIVE**
	0.35 -- 1.0 IU / ml	**LOW**
	1.1 -- 5.0IU / ml	**MODERATE**
	5.1 -- 25.0 IU / ml.	**HIGH**

< 0.35	**No specific antibodies detected.**
0.35 -- 1.0 IU / ml	**Low antibody titer, existing sensitisation, frequently with clinical symptoms in the upper range of class**
1.1 -- 5.0	**Significant antibody titer, clinical symptoms usually present**
15.1 -- 25.0	**High antibody titer, almost always with clinical symptoms.**

Allergen Detected

Food Veg	**MSG (AJINOMOTO),STRAW BERRY,YEAST**
Inhalants	**INSECT,JUTE DUST,WOOD DUST**
Contact	
Drugs	

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SAMPLE : Serum

FOOD VEG

ALMOND			0.25	ANISEED			0.25
APPLE			0.22	ARECANUT			0.21
BAJARI			0.30	BANANA			0.18
BARLEY			0.17	BLACK PEPPER			0.28
BRINJAL			0.25	CABBAGE			0.13
CAPSICUM			0.23	CARDIMUM			0.29
CARROT			0.14	CASHEWNUT			0.19
CAULIFLOWER			0.30	CHEESE			0.16
CHILLY			0.24	CINNAMON			0.26
CLOVE			0.27	COCOA			0.16
COCONUT			0.15	COFFEE			0.20
CORIANDER			0.23	COTTON SEED			0.14
CUCUMBER			0.13	CUMMIN			0.22
CURD			0.17	DAL ARHAR			0.28
DAL CHANA			0.27	DAL MASOOR			0.23
DAL MOONG			0.22	DAL RAJMA			0.15
DAL URAD			0.10	DATES			0.26
EGGS			0.13	FENUGREEK			0.15
GARLIC			0.16	GINGER			0.10
GRAPES			0.21	GROUNDNUT			0.19
GUVAVA			0.12	HONEY			0.16

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SAMPLE : Serum

JAGGERY			0.18	KATHA			0.27
LADY S FINGER			0.26	LEMON			0.10
MAIZE			0.15	MANGO			0.10
MELON			0.11	MILK			0.18
MSG (AJINOMOTO)		+	0.69	MUSHROOM			0.14
MUSTARD			0.11	NUTMEG			0.17
OAT			0.23	ONION			0.22
ORANGE			0.22	PAPAYA			0.15
PEAS			0.19	PINEAPPLE			0.29
PISTA			0.10	POTATO			0.30
RADISH			0.20	RICE			0.28
SALT			0.25	SESAME			0.28
SOYABEAN			0.12	SPINACH			0.10
SQUASH			0.16	STRAW BERRY		+	0.87
SUGAR			0.21	SUNFLOWER			0.30
TAMARIND			0.22	TEA			0.15
TOBACCO			0.29	TOMATOES			0.12
TURMERIC			0.22	VINEGAR			0.14
WALNUT			0.28	WHEAT			0.20
YEAST		+	0.87				

INHALANTS

ACACIA ARABICA			0.18	ADHATODA VASICA			0.20

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SAMPLE : Serum

ALTERNARIA TENUIS			0.23	AMARANTHUS			0.28
ARGEMONE MEXICANA			0.12	ARTEMISIA SCOPARIA			0.23
ASPERGILLUS			0.18	ASPERGILLUS NIGER			0.20
ASPERGILLUS TAMARIE			0.11	AZADIRACHTA INDICA			0.28
BERMUDA GRASS			0.18	BRASSICA CAMPESTRIS			0.29
BUFFALO DANDER			0.22	CANDIDA ALBICANS			0.23
CANNABIS SATIVA			0.27	CARICA PAPAYA			0.28
CASSIA FISTULA			0.14	CAT DANDER			0.29
CEDRUS DEODARA			0.27	CENCHRUS CILIARIS			0.28
CHAK POWDER			0.13	CHENOPODIUM ALBUM			0.16
CLADOSPORIUM HERB			0.26	COCKROACH DUST			0.19
COCOS NUCIFERA			0.20	COTTON DUST			0.20
COTTON FIBERS			0.16	COW DANDER			0.16
CRICKET DUST			0.22	CYNODON DACTYLON			0.22
CYPERUS ROTUNDUS			0.25	DAISY			0.17
DANDELION			0.19	DOG DANDER			0.11
ETHYLENE OXIDE			0.30	EUCALYPTUS			0.30
FEATHER DUST			0.12	FLOUR MILL DUST			0.26
GOAT DANDER			0.14	GRAVEL DUST			0.12
HAY DUST			0.16	HOLOPTELEA INT			0.11
HONEY BEE			0.26	HORSE DANDER			0.27
HOUSE DUST MITE			0.13	HOUSE FLY			0.15

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SAMPLE : Serum

HOUSEDUST			0.13
INSECT		+	1.65
JEERA DUST			0.24
JOWAR DUST			0.10
JUNIPERUS COMMUNIS			0.15
KIGELIA PINNATA			0.18
MOSQUITO			0.29
NEUROSPOR SPP			0.18
PARTHENIUM			0.14
PHOENIX DACTYLIFERA			0.27
RAGWEED			0.14
RICE DUST			0.27
RYE GRASS			0.21
SORGHUM VULGARE			0.11
SUAEDA FRUTICOSA			0.17
TYPHA ANGUSTATA			0.25
WOOD DUST		+	0.87
ZEA MAYS			0.12

IMPERATA CYLINDRICA			0.19
IPOMOEA FISTULOSA			0.18
JOHNSON GRASS			0.18
JULIFLORA			0.10
JUTE DUST		+	1.52
LAWSONIA INERMIS			0.22
MUGWORT			0.17
PAPER DUST			0.11
PENICILLIUM SPP			0.29
PINE			0.12
RHIZOPUS NIGRICANS			0.29
RICINUS COMMUNIS			0.25
SHEEP WOOL			0.13
STRAW DUST			0.15
TEA LEAVES			0.21
WHEAT DUST			0.26
XANTHIUM STERMARLUM			0.25

CONTACT

ALCOHOL			0.17
BONE CEMENT PMMA			0.24
DETERGENT			0.23

ASH-LEAF ELDER			0.25
CAUSTIC SODA			0.10
HAIR DYE			0.28

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SAMPLE : Serum

LATEX			0.26	LEATHER			0.16
LIME STONE			0.17	NICKEL			0.16
NYLON FIBERS			0.13	PAINTS			0.24
PERFUME			0.22	PLASTIC			0.28
POLYESTER			0.21	POWDER			0.20
SILK			0.17	SMOKE			0.12
SMOKING(TOBACCO)			0.13	SOAP			0.15
SULFUR			0.25	WOOL MIX			0.26
YARN FIBRE			0.15				

DRUGS

AMOXICILLIN			0.18	AMPICILLIN			0.28
ARTICAIN			0.24	ASPIRIN			0.10
BRUFEN			0.12	CEPHALOSPORIN			0.11
CIPROFLOXACIN			0.13	CLOXACILLIN			0.17
DICLOFENAC			0.22	DOXYCYCLIN			0.10
EPINEPHRINE			0.24	ERYTHROMYCIN			0.11
INSULIN (HUMAN)			0.29	KETOPROFEN			0.13
METAMIZOL			0.15	NIMESULIDE			0.27
NORFLOXACIN			0.16	OFLOXACIN			0.23
OXACILLIN			0.26	PARACETAMOL			0.15
PENICILLIN			0.25	PHENYLBUTAZONE			0.17
PIROXICAM			0.17	PROCAIN			0.25

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SAMPLE : Serum

STREPTOMYCIN			0.18	SULPHA			0.13
TETRACYCLINE			0.18	TETRAZINE			0.14
THEOPHILLINE			0.29	TINIDAZOLE			0.28
TRIMETHROPRIM			0.18	XYLOCAIN			0.10

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REPORT

PT Name	: **MRS.MANAMANI DARMA**	**Age :** 56 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. R.N HOSPITAL**	**Registered on :** 23-08-2026 02:12 PM	RN DIAGNOSTIC
Reg No	: **HL7601823773 / MH591**	**Received on** : 23-08-2026 03:52 PM	
Barcode	: **HEN121948**	**Reported on** : 24-08-2026 01:00 PM	
INV	: Antidiuretic Hormone (ADH)		**SAMPLE :** Serum

ANTIDIURETIC HORMONE (ADH)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
ANTIDIURETIC HORMONE (ADH) Method: CLIA	2.36	pg/ml	1.0 - 5.0

Clinical significance :

Abnormal results: Patients who are dehydrated, who have a decreased amount of blood in the body (hypovolemia), or who are undergoing severe physical stress (e.g., trauma, pain or prolonged mechanical ventilation) may exhibit increased ADH levels. Patients who are overly hydrated or who have an increased amount of blood in the body (hypervolemia) may have decreased ADH levels.

Other conditions that cause increased levels include SIADH, central nervous system tumors or infection, or pneumonia.

Principle: The purpose of ADH is to control the amount of water reabsorbed by the kidneys. Water is continually being taken into the body in food and drink, as well as being produced by chemical reactions in cells. Water is also continually lost in urine, sweat, feces, and in the breath as water vapor. ADH release helps maintain the optimum amount of water in the body when there is an increase in the concentration of the blood serum or a decrease in blood volume. Physical stress, surgery, and high levels of anxiety can also stimulate ADH.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **BUSHRA**
Ref By : **DR. BMC HOSPITAL**
Reg No : **HL7101830995 / MH552**
Barcode : **HEN110341**
INV : Anti Ds DNA Antibody

Age : 21 Year | **Sex :** Female
Registered on : 30-08-2026 11:57 AM
Received on : 30-08-2026 02:57 PM
Reported on : 30-08-2026 10:24 PM

SAMPLE COLLECTED AT :
AMISHA LAB

SAMPLE : Serum

ANTI DS DNA ANTIBODY

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Anti- Ds Dna Antibody Method: ELISA	0.16	Ratio	0 - 0.9 Normal : < 0.9 Borderline : 0.91 - 1.2 High : > 1.2

Clinical significance :
dsDNA antibodies are detectable in approximately 85% of patients with untreated SLE, and are rarely detectable in other connective tissue diseases. Weakly positive results caused by low-avidity antibodies to dsDNA are not specific for SLE and can occur in a variety of diseases.

Testing for IgG antibodies to dsDNA is indicated in patients who have a positive test for antinuclear antibodies (ANA) along with signs and symptoms that are compatible with the diagnosis of SLE.(2) If the ANA test is negative, there is no reason to test for antibodies to dsDNA.(2)

The levels of IgG antibodies to dsDNA in serum are known to fluctuate with disease activity in lupus erythematosus, often increasing prior to an increase in inflammation and decreasing in response to therapy.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **MANPREET KAUR**
Ref By : **DR. SELF**
Reg No : **HL3301831094 / MH677**
Barcode : **HEI057044**
INV : Complement 3 (C3)

Age : 29 Year | **Sex :** Female
Registered on : 30-08-2026 12:45 PM
Received on : 30-08-2026 05:37 PM
Reported on : 31-08-2026 07:28 AM

SAMPLE COLLECTED AT :
NEO

SAMPLE : Serum

COMPLEMENT 3 (C3)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Complement c3	121.5	mg/dl	90.00 - 180.00
Method: (Immunoturbidimetry)			

Comments

The complement system is an integral part of the body's immune defenses. The primary complement pathway consists of recognition (Clq, Clr, Cls), activation (C4, C2, C3), and attack (C5, C6, C7, C8, C9) mechanisms with respect to their role in antibody-mediated cytolysis.

The complement system can be activated via immune complexes, and the alternative pathway (properdin pathway), which is activated primarily by foreign bodies such as microorganisms.

C3 activation involves cleavage by C3 convertase into C3a and C3b. When immune complexes are not involved, the alternate method of complement activation initiates the reactant sequence at C3, bypassing C1, C4, and C2.

Severe recurrent bacterial infections occur in patients with homozygous C3 deficiency and in those patients with low levels of C3 secondary to the absence of C3b activator.

Decreased C3 may be associated with acute glomerulonephritis, membranoproliferative glomerulonephritis, immune complex disease, active systemic lupus erythematosus, septic shock, and end-stage liver disease.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **FARZANA QURESHI**
Ref By : **DR. BMC HOSPITAL**
Reg No : **HL3001824396 / MH552**
Barcode : **HEN186769**
INV : Endomysial Antibody IgA

Age : 42 Year | **Sex :** Female
Registered on : 24-08-2026 12:44 PM
Received on : 25-08-2026 05:50 PM
Reported on : 25-08-2026 08:58 PM

SAMPLE COLLECTED AT :
AMISHA LAB

SAMPLE : Serum

ENDOMYSIAL ANTIBODY IGA

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Endomysial Antibody IgA	27.0	U/mL	0.00 - 150.00

Clinical significance :
The finding of IgA-endomysial antibodies (EMA) is highly specific for dermatitis herpetiformis or celiac disease.

The titer of IgA-EMA generally correlates with the severity of gluten-sensitive enteropathy.

If patients strictly adhere to a gluten-free diet, the titer of IgA-EMA should begin to decrease within 6 to 12 months of onset of dietary therapy.

Occasionally, the staining results cannot be reliably interpreted as positive or negative because of strong smooth muscle staining, weak EMA staining or other factors; in this case, the results will be recorded as "indeterminate." In this setting, further testing with measurement of TTGA / Tissue Transglutaminase (tTG) Antibody, IgA, Serum and IGA / Immunoglobulin A (IgA), Serum levels are recommended.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **SANGEETA KOILKAR**
Ref By : **DR. SELF**
Reg No : **HL3801829851 / MH031**
Barcode : **HEN199498**
INV : Fluid Routine Examination

Age : 48 Year | **Sex :** Female
Registered on : 29-08-2026 09:38 AM
Received on : 29-08-2026 04:30 PM
Reported on : 31-08-2026 11:33 PM

SAMPLE COLLECTED AT :
SHRIKANT MURCHAVDE

SAMPLE : Body Fluid

FLUID ROUTINE EXAMINATION

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
QUANTITY Method: .	05	ML	. - .
COLOUR Method: .	PALE YELLOW	.	. - .
SAMPLE TYPE Method: .	SYNOVIAL FLUID	.	. - .
GRAM STAIN Method: .	NO MICRO-ORGANISM SEEN	.	. - .
AFB STAIN Method: .	NOT SEEN	.	. - .
OTHERS Method: .	.	.	. - .
RBC'S	NIL	.	. - .
LYMPHOCYTES Method: .	80	%	. - .
POLYMORPHS Method: .	20	%	. - .
Total Leucocyte Count(TLC) Method: .	326	.	. - .
CHLORIDES	**106.50**	mmol/L	110 - 135
PROTEINS	2.50	g/dL	2.5 EXUDATES
GLUCOSE	**100.6**	mg/dl	70 - 100
SPECIFIC GRAVITY Method: .	1.025	.	1.020 EXUDATES
PH	7.0	.	7 - 8
COAGULUM	ABSENT	.	. - .
APPEARANCE Method: .	SLIGHTLY HAZY	.	. - .

Seen are inflammatory cells comprising predominantly lymphocytes (80%) along with neutrophils (20%). Background staining appears pink and fine to coarsely granular.

Impression- Chronic Inflammatory process.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **BABAN YADAV**	**Age :** 57 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. MILIND PARADKAR**	**Registered on :** 31-08-2026 04:30 PM	
Reg No	: **HL7701832383 / MH399**	**Received on** : 31-08-2026 05:34 PM	
Barcode	: **HEJ044941**	**Reported on** : 01-09-2026 11:38 PM	
INV	: Gene Xpert (CB NAAT) SPUTUM		**SAMPLE :** SPUTUM

GENE XPERT (CB NAAT) SPUTUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Mycobacterium Tuberculosis Method: Semi - Nested Real Time PCR	NOT DETECTED	.	. - .
Rifampicin Method: -	.	-	- - -

Clinical significance :
Result Interpretation:

- **If result is TB negative: This indicates no tuber culosis (TB). However, if the clinical picture is strongly** suggestive of TB, a repeat test can be suggested by your referring doctor.

- If result is TB positive, and Rifampicin resistan ce negative: This indicates tuberculosis that can be treated with commonly used drugs. Please seek assistance from the referring doctor for treatment of tuberculosis, which is a highly curable disease.

If result is Rifampicin resistance positive and you have previously taken TB treatment: This indicates drug resistance - TB that is difficult to treat with commonly used drugs. Please seek assistance from the referring doctor for treatment of multidrug resistant tuberculosis (MDR TB) which requires specialized management,including additional drug-susceptibility testing.

- If result is Rifampicin resistance positive and you have NOT previously taken TB treatment: This may indicate drug resistance, but requires confirmation. Please seek a second test from an accredited laboratory to confirm the presence of multidrug resistant tuberculosis (MDR TB) which requires specialized management.

You are entitled to FREE TB drugs from specific government TB centers and hospitals.Details are available on www.ipaqt.org.

Clinical Background:
GeneXpert MTB/RIF, is a rapid automated molecular test for diagnosis of Mycobacterium tuberculosis (MTB) and resistance to rifampicin (RIF) in clinical specimens.

- It targets rpoB gene (hot spot region) which is critical for identifying Rifampicin resistance mutations.

MDR TB is defined as TB due M.tb complex,resistant to both Isoniazid & Rifampicin. Rifiampicin acts as a surrogate marker for MDR TB as 90% of rifampicin resistant strains are also isoniazid resistant.

~~End of report~~

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REPORT

PT Name : **BHARTI NANDA**
Ref By : **DR. ARJUN BHANUSHALI**
Reg No : **HL2301827856 / GUJ011**
Barcode : **HEN197808**
INV : Ketone (Acetone,Hydroxybutyrate),Urine

Age : 60 Year | **Sex :** Female
Registered on : 26-08-2026 11:29 PM
Received on : 27-08-2026 07:10 AM
Reported on : 28-08-2026 07:55 AM

SAMPLE COLLECTED AT :
NEELPATH DIAGNOSTIC ™

SAMPLE : Urine

KETONE (ACETONE,HYDROXYBUTYRATE),URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Urine for ketone	2.69	mg/dl	0.0 - 5.0

Clinical significance :

Ketones are usually absent in urine. Their presence most likely indicates that the body is using fats rather than carbohydrates for energy. High levels of ketones may be present in the urine of individuals with uncontrolled diabetes because the body's ability to metabolize carbohydrates is defective.

The presence of ketones in the urine is a valuable aid to managing and monitoring individuals with diabetes mellitus. Ketonuria is an indication that the insulin dose needs to be increased. Electrolyte imbalance and dehydration may occur when ketones accumulate in the blood. If these conditions are not corrected by adjusting the dose of insulin, the patient may develop ketoacidosis and ultimately diabetic coma.

Low levels of ketones may also be detected in the urine during conditions of physiological stress such as fasting, rapid weight loss, frequent strenuous exercise or prolonged vomiting. The presence of ketones in these situations is due to either inadequate intake of carbohydrates or increased loss of carbohydrate

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **SATYAWATI GURAV**
Ref By : **DR. GMC**
Reg No : **HL5301830810 / MH004/7/ORAS01**
Barcode : **HEN128261**
INV : Lactate Dehyderogenase (LDH) FLUID

Age : 67 Year | **Sex :** Female
Registered on : 30-08-2026 08:22 AM
Received on : 30-08-2026 11:40 AM
Reported on : 30-08-2026 04:27 PM

SAMPLE COLLECTED AT :
SHUBHAM LAB

SAMPLE : Body Fluid

LACTATE DEHYDEROGENASE (LDH) FLUID

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LDH FLUID Method: IFCC	**421**	U/L	140.0 - 280.0

NOTE-
Reference range in LDH is not established for body fluids. Physician to correlate clinically.

Clinical significance :
Lactate dehydrogenase (LDH) activity is present in all cells of the body with the highest concentrations in the heart, liver, muscle, kidney, lung, and erythrocytes.

fluid is normally present within the pleural cavity surrounding the lungs, serving as a lubricant between the lungs and inner chest wall. effusion develops when the fluid cavity experiences an overproduction of fluid due to increased capillary hydrostatic and osmotic pressure that exceeds the ability of the lymphatic or venous system to return the fluid to circulation. Laboratory-based criteria are often used to classify fluid effusions as either exudative or transudative. Exudative effusions form due to infection or inflammation of the capillary membranes allowing excess fluid Patients with these conditions benefit from further investigation and treatment of the local cause of inflammation. Transudative effusions form due to systemic conditions such as volume overload, end stage renal disease, and heart failure that can lead to excess fluid accumulation in the cavity. Patients with transudative effusions benefit from treatment of the underlying condition.(1) Measurement of LDH in body fluids is primarily indicated to aid in the differentiation of transudative and exudative effusions as LDH activity is considered an indicator of the extent of inflammation.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **MR. NAVEEN KUDARI**
Ref By : **DR. DR. SHUBHA RAO**
Reg No : **HL6401820486 / MH303**
Barcode : **HEN161926**
INV : Salmonella Typhi (TYPHI DOT IgG)

Age : 40 Year | **Sex :** Male
Registered on : 20-08-2026 01:39 PM
Received on : 20-08-2026 02:16 PM
Reported on : 20-08-2026 05:18 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

SALMONELLA TYPHI (TYPHI DOT IGG)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Typhi D Method: ELISA	**1.07**	OD Ratio	0.00 - 0.90

Clinical significance :
The Typhidot is easy to perform, and requires no special equipment or training of staff for interpretation of results. It will be a useful complementary test to blood culture and the Widal test in the diagnosis of typhoid fever.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **SANVI BHOYE**
Ref By : **DR. SUMANGAL HOSPITAL**
Reg No : **HL6101823605 / MH004/012/W86**
Barcode : **HDR207053**
INV : sickle Cell

Age : 3 Year | **Sex :** Female
Registered on : 23-08-2026 12:31 PM
Received on : 25-08-2026 01:40 PM
Reported on : 26-08-2026 12:14 AM

SAMPLE COLLECTED AT :
AAROGYAM PATH LAB,JAWAHR

SAMPLE : EDTA Blood

SICKLE CELL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
sickle cell	POSITIVE	.	. - .

Method: .

Clinical significance :

Yes. A person can have a mixture of normal and faulty hemoglobin in their red blood cells without having sickle cell disease. This condition is called "sickle cell trait." People with sickle cell trait have enough normal hemoglobin in their red blood cells to prevent the cells from sickling. One in 12 African Americans in the United States has sickle cell trait.

It's important to remember that people with sickle cell trait do not have sickle cell disease. They also usually do not develop sickle cell disease, except in unusual circumstances. However, people with sickle cell trait can genetically pass the trait to their children. If two people with sickle cell trait have children together, there is a 1 in 4 chance that their children will have sickle cell anemia

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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HY PATHO LAB
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HY PATHO LAB RANCHI

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REPORT

PT Name	: **ANKITA RAI**	**Age :** 36 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. DR.NAVNEET GUPTA**	**Registered on :** 28-08-2026 11:06 AM	ARYA DIGNOSTIC LAB
Reg No	: **HL3701828989 / GUJ01**	**Received on** : 06-09-2026 06:26 PM	
Barcode	: **FH982479**	**Reported on** : 10-09-2026 12:31 PM	
INV	: Stone Analysis Gall Bladder		**SAMPLE :** Stone

STONE ANALYSIS GALL BLADDER

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

BIOPSY-15137/26

NOS- GALL BLADDER MASS FOR HPE.

GROSS -RECEIVED A GRAY BROWN MASS MEASURING 3X2 CM , ONE BIT GIVEN IN ONE BLOCK.

MICROSCOPIC PICTURE- SECTION STUDIED FROM GALL BLADDER MASS SHOWS SHEETS OF CLEAR CELLS IN AN ALVEOLAR ARRANGEMENT SEPARATED BY BLOOD VESSELS, SIMILAR STROMAL MIXED INFLAMMATORY INFILTRATES WITH PLASMA CELLS AND NECROTIC DEBRIS.

IMPRESSION- SUGGESTIVE OF CHRONIC CHOLECYSTITIS.

COMMENT -KINDLY CORRELATE CLINICALLY .

~~End of report~~

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REPORT

PT Name	: **KOMAL SURAJ GAWADE**	**Age :** 27 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. MAYUR MORE**	**Registered on** : 30-08-2026 04:23 PM	
Reg No	: **HL6401831428 / MH428**	**Received on** : 30-08-2026 07:58 PM	
Barcode	: **HEJ067570**	**Reported on** : 30-08-2026 08:41 PM	**SAMPLE :** Serum
INV	: HY ANC ADVANCE		

VDRL (RAPID)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Vdrl Method: .	NEGATIVE	.	. - .

~~End of report~~

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